Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

OPERATIONAL GUIDELINE

On Call Duties & Arrangements

YG OPG 010 14TH AUGUST 2025

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Key Points

- The on-call team should work as a team to support each other and provide safe and effective care for all of the patients under our care.
- Everyone should be aware of their primary responsibilities.
- You should not take on tasks that will make you unavailable to attend your primary responsibilities unless it is a clinical emergency.

Introduction.

The purpose of this guideline is to explain the roles, responsibilities and expectations of the on-call anaesthetic and critical care doctors.

It is important to note that this is guidance, and it cannot cover every situation. The overwhelming priority is that the on-call team should work as a team to support each other and ensure that they maintain safe and effective care to all of the patients under our care.

Roles.

Daytime.

1. Consultant on call 08:00-20:30 Page 2
2. ICU consultant on call 08:00-20:30 Page 2
3. 1st on call resident 08:00-20:30 Page 3
4. ICU resident 08:00-20:30 Page 4
5. Obstetric resident 08:00-20:30 Page 5
6. Trauma anaesthetist 08:00-17:45 Page 6
7. Starred consultant (normal working weekday only) 08:00-17:00 Page 6
8. Purple star consultant (where required to support labour ward) Page 6

Night.

1. Consultant on call resident 20:00-23:00 non-resident 23:00-08:00 Page 2
2. 2nd on (non-resident consultant) 20:00-08:00 Page 6
3. 1st on call resident 20:00-08:30 Page 3
4. ICU resident 20:00-08:30 Page 4
5. Obstetric resident 20:00-08:30 Page 5

Consultant on call

Daytime 08:00-20:30

- Carry bleep 77642 and ensure you are contactable during the on call. Meet with the residents in the morning. Attend the CEPOD team brief at 09:00 in theatre 4/recovery
- Primary responsibility is supervising the 1st on call and anaesthetic cover for the emergency theatre list (CEPOD/theatre 4).
- Also responsible for responding to appropriate requests for anaesthetic consultant support

Night time resident 20:00-23:00

As above plus

- Supporting the ICU resident with review of critical care patients
- Supporting the obstetric resident with management of the labour ward
- Call in the second on call consultant is workload or site safety deems it necessary

Night time on call 23:00-08:00

- Can go home but must be contactable and be withing 30 minutes of the hospital
- Respond to calls for advice or support form any of the anaesthetic/ICU/obstetric residents
- Take consultant referrals for ICU
- Call in the second on call consultant if you are already on site and workload or site safety deems it necessary

ICU consultant on-call

Daytime 08:00-20:30

- Carry bleep 77103 and ensure you are contactable during the on call
- Attend ICU hand over in the morning
- Primary responsibilities are supervising the ICU residents and leading the ward round for critical care patients.
- Overall, in charge of clinical decisions for all critical care patients.
- Attend handover at 20:00 to the night team.
- Other responsibilities include (but not limited to):
- Responding to appropriate requests for consultant intensive care support.
- Take referrals for critical care.
- Support other areas where necessary – e.g. - if anaesthetic consultant is busy.

1st on call resident

Daytime 08:00-20:30

- Carry bleep 77063 and ensure you are contactable during your on-call.
- Receive handover from the outgoing resident.
- Check emergencies booked for theatre and pre-op assess any patients that have not yet been reviewed.
- Complete machine checks for theatre 4 and pre-draw emergency drugs.
- Meet with the consultant at the earliest opportunity to discuss cases
- Attend the CEPOD team brief at 09:00 in theatre 4/recovery
- Your primary responsibility is the emergency list in theatre 4.
- Other responsibilities include (but not limited to):
    - Responding to anaesthetic emergencies in theatres, recovery, PACU or the wards.
    - Responding to requests from the acute pain team or the wards to assist with acute pain issues. If available - Responding to cardiac arrest calls, MET calls, trauma calls, paediatric emergencies, major haemorrhage and any requests for anaesthetic support. If there are emergencies waiting to be done in theatre you should free yourself as soon as practical.
    - If you are not available to attend an emergency call (e.g. because you are in theatre) then it is YOUR responsibility to contact someone else to go in your place. Either the ICU resident, the on-call consultant or other. Do not just ignore the call.
- Call the consultant on call whenever you need support or are not confident that you can carry out these duties safely.

Night time 20:00-08:30

As above plus

- If no emergencies in theatre, then present yourself to ICU and help the ICU resident with patient reviews and jobs.
- You can also assist the obstetric resident with emergencies if you have passed initial assessment of obstetric competencies (IAOC). If you have not passed IAOC you can attend to assist emergencies to support the obstetric resident at their request (e.g. with cannulation) but be clear what you can and cannot do.

ICU resident

Daytime 08:00-20:30

- Carry bleep 77550 and ensure you are contactable during your on-call.
- Attend handover meeting and receive handover from the outgoing resident.
- Review the critical care patients, attend daily ward round, complete tasks arising from daily ward round.
- Check emergency intubation drugs.
- Be available to review any patients or do any procedures as requested by the critical care nurses.
- Review patients referred for critical care and discuss with the ICU consultant.
- If available - Responding to cardiac arrest calls, MET calls, trauma calls, paediatric emergencies, major haemorrhage and any requests for critical care support.
- If you are not available to attend an emergency call (e.g. because you are doing a critical care procedure or with an unstable patient) then contact someone else to go in your place.
- Call the ICU consultant whenever you need support or are not confident that you can carry out these duties safely.

Night time 20:00 – 08:30

As above, but note there is only one consultant at night (On-call consultant) covering critical care, theatres and maternity.

Obstetric resident

Daytime 08:00-20:30

- Carry bleep 77551 and ensure you are contactable during your on-call.
- Attend labour ward for the obstetric anaesthetists' handover at 08:00 and the multidisciplinary obstetric team handover at 08:30.
- Your primary responsibility is to provide anaesthetic support to the obstetric team. You should be immediately available to attend the obstetric unit 24 hours per day, and must therefore have no other responsibilities outside obstetrics.
- Epidural requests: as per the OAA guidelines, the time from being informed that a woman is requesting an epidural and ready to receive one until attending the mother should not normally exceed 30 minutes. This period should only exceed one hour in exceptional circumstances. *These are guidelines and it is accepted that there will be rare circumstances when we cannot safely meet these targets.*
- Other responsibilities include:
    - Check the anaesthetic machines in obstetric theatre and anaesthetic room.
    - Replace GA and emergency drugs *(daytime only)*
    - Complete follow up reviews *(daytime only)*
- Call the consultant covering labour ward (or consultant on call) whenever you need support or are not confident that you can carry out these duties safely.
- In addition, call the consultant on-call for major obstetric and anaesthetic emergencies e.g.:

    PPH >1500mls with ongoing bleeding
    Eclampsia / severe pre-eclampsia
    Placental abruption / previa / accreta
    Failed intubation
    Total spinal / LA toxicity
    Amniotic fluid embolism
    Complications of epidural such as suspected haematoma / PDPH

Night time 20:00-08:30

As above.

- If there are no duties to do on labour ward you should assist the ICU resident / 1st on-call but must remain free to attend labour ward immediately in an emergency.
- In the rare circumstance that there is an obstetric emergency while you are already committed to an obstetric emergency theatre case then call the consultant on call. You can also call the ICU/1st on call residents to support you if they are available. Be mindful that they may not be obstetric trained or may not have previously covered obstetrics in YG, but they may provide extra assistance until the consultant comes.

Out of hours epidural requests

If you are unable to attend within 30 minutes of a woman requesting an epidural, call your supervising consultant (08:00-17:00) or the on-call consultant (17:00-23:00) to help. If the consultant is non-resident or not available then siting the epidural should be delayed until you are available.

If you are unlikely to be able to attend within an hour and there is clinical justification why it cannot wait then you can request the ICU resident or 1st on call (if obstetric trained) to undertake the epidural. They should only attend if the likelihood of them being needed on ICU/in theatres is deemed low. The ICU resident/1st on call should not routinely be called to undertake epidurals (even If they have are obstetric trained).

2nd on call consultant

Non-resident on call 20:00-08:00

- You will only be called by the 1st on call consultant if the clinical workload becomes unsafe and there is a threat to life, limb or eyesight
- You must be contactable by personal mobile and able to respond to a request to attend the hospital within 60 minutes
- In the rare circumstance that the 1st on call consultant has to replace one of the resident on call anaesthetists (e.g due to sickness or transfer) then you effectively become the on call consultant. You do not need to attend unless requested by the 1st on call consultant.

Trauma Anaesthetist (theatre 2)

Resident 08:00-17:45

- You are primarily responsible for the orthopaedic trauma list
- Attend the trauma meeting at 08:00
- Call the consultant on call if you need support, a break or are not confident that you can carry out your duties safely.

Starred consultant

Resident 08:00-17:00

- Normally allocated to a list with another anaesthetist capable of being left along with that list.
- You should ensure you are contactable. If you leave your allocated theatre, make sure the anaesthetist you are working with knows where you are and how to contact you.
- You are essentially the daytime second on consultant, there to provide assistance to the 1st on call consultant if there is an urgent clinical need and they are already committed to another clinical emergency.
- Always check in with the on-call consultant if you have finished your theatre list early.

Purple star consultant

- Carry the 77170 bleep
- You are identified on the rota by a purple star and you are the named consultant to support a resident doctor covering the labour ward (when a consultant is not available to cover directly).
- You will be allocated to a list with another anaesthetist capable of being left alone with that list.
- If you leave your allocated theatre, make sure the anaesthetist you are working with and the anaesthetist on labour ward knows where you are and how to contact you.